

MIST. POSSIT**PRODUCT DESCRIPTION**

A yellow solution, free from any foreign particles. Colour: Yellow.

Each 10 mL solution contains:

Potassium Citrate : 3.00 g

Citric Acid Monohydrate : 0.50 g

Preservative:

Methyl Paraben : 0.04% w/w

Potassium Sorbate : 0.15% w/w

PHARMACODYNAMICS

Potassium citrate and citric acid solutions are known as systemic and urinary alkalinizers which provide symptomatic relief of dysuria.

PHARMACOKINETICS

Potassium Citrate is absorbed and the citrate is metabolised to bicarbonate. Citric acid is metabolised to carbon dioxide and water. Oxidation is virtually complete with less than 5% of citrate being excreted unchanged in the urine.

INDICATIONS

It is indicated for relief of discomfort in mild urinary tract infections and alkalinisation of urine.

RECOMMENDED DOSE

Adult:

10 mL, three times daily.

Children (1-6 years):

5 mL, three times daily.

Dilute with a full glass of water. To be taken with or after meals.

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

Contraindicated in hyperkalaemia, renal dysfunction, ventricular arrhythmics and Addison's Disease.

WARNINGS & PRECAUTIONS

Large doses may cause hyperkalemia & alkalosis, especially in the presence of renal disease. Should be used with caution by patients with low urinary output unless under the supervision of a physician.

As with all liquids containing a high concentration of potassium, patients should be directed to dilute adequately with water to minimize the possibility of gastrointestinal injury associated with the oral ingestion of concentrated potassium salt preparations; and preferable, to take each dose after meals to avoid saline laxative effect.

Potassium citrate mixture provides symptomatic relief only and is not anti-bacterial. Effective anti-bacterial therapy should be co-prescribed. It should be used with caution when renal impairment, cardiac dysfunction or hypotension is present.

Use with caution in the elderly.

INTERACTION WITH OTHER MEDICAMENTS

Concurrent administration of potassium-containing drugs, potassium sparing diuretics or other drugs that increase potassium levels (e.g. ACE inhibitors, ciclosporin, aliskiren) may lead to hyperkalaemia. May interact with cardiac glycosides. Citrates alkalise the urine and thus may alter the urinary excretion of a number of drugs. This may lead to increased renal clearance of acidic drugs, such as salicylates, tetracyclines and barbiturates, and prolongation of the half-life of basic drugs, such as sympathomimetics and stimulants. Particularly noteworthy is the diminished anti-bacterial activity of nitrofurantoin and methenamine.

PREGNANCY & LACTATION

No adverse effects are anticipated at recommended doses when used for initial symptomatic relief only. Treatment with potassium citrate mixture is adjunctive and secondary to anti-bacterial treatment of urinary tract infection.

SIDE EFFECTS

Abdominal discomfort, vomiting, diarrhoea, loose bowel movements, nausea, gastrointestinal disturbances and mild diuresis. High levels of potassium in blood may cause muscle weakness, irregular heart action and paralysis.

Potentially Fatal: Hyperkalaemia.

SYMPTOMS & TREATMENT OF OVERDOSAGE**Symptoms:-**

Overdosage is accompanied by nausea, vomiting, abdominal pain and symptoms due to hyperkalaemia and metabolic acidosis. Fluid and electrolyte balance together with ECG should be closely monitored. Hyperkalaemia is manifested by increased serum potassium concentration and ECG changes, while late manifestations include muscle paralysis and cardiovascular (CV) collapse from cardiac arrest.

Treatment:-

Symptomatic and supportive. Moderate to severe hyperkalaemia is a medical emergency requiring prompt correction. Eliminate medications containing potassium, agents with potassium sparing properties (e.g. potassium-sparing diuretics, ACE inhibitors, NSAIDs) and food with high potassium content (e.g. almonds, beans, milk, salmon). May give intravascular (IV) calcium gluconate if the patient is at no risk or low risk of developing digitalis toxicity. Administer IV dextrose 10% containing 10-20 U of crystalline insulin per 1,000 mL at a rate of 300-500 mL/hr. Acidosis may be corrected with IV Na bicarbonate. May perform haemodialysis or peritoneal dialysis.

INSTRUCTIONS FOR USE

Dilute with a full glass of water. To be taken with or after meals.

PACK SIZE

Plastic Container of 120mL.

STORAGE CONDITIONS

Store below 30°C, away from direct light.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

REGISTRATION NO

MAL20001569X

**KEEP MEDICINES OUT OF REACH OF CHILDREN
JAUHI UBAT-UBATAN DARI KANAK-KANAK**

For further information, please consult your doctor or your pharmacist.

Revision No 05

Revision Date : 11 Dec 2025

Lt-177.06 PRODUCT REGISTRATION HOLDER AND MANUFACTURER

INSPIRAPHARMA SDN. BHD. (200401023395)

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